▼ VIEWPOINT

PUTTING AN END TO HEPATITIS B

Screening, vaccination and destigmatisation are essential to eliminating a disease that affects 29 million Indians



BY DR SHIV K. SARIN

Every 30 seconds, globally, someone dies from hepatitis-related liver infections—amounting to 1.3 million deaths annually as per WHO 2024 estimates. The Hepatitis B Virus (HBV) alone contributes to nearly 900,000 of these deaths, mostly due to cirrhosis and liver cancer. That's more than malaria, and approaching tuberculosis levels. The virus is carried by an estimated 29 million Indians—the second-highest number globally. And yet, despite having a protective vaccine and effective antivirals for over three decades, we have failed to eliminate it. Why?

The painful truth: doctors have failed to engage society. We treated HBV as a medical issue, not a societal one like COVID-19, HIV or tuberculosis. For most people, the infection lasts less than six months. For others, it can get chronic and seriously damage the liver. Shockingly, less than 10 per cent of infected individuals are diagnosed; stigma around sexual transmission of the virus has led to under 5 per cent receiving treatment. This ignorance is far from bliss. The HBV can spread through blood, semen, saliva and other bodily fluids, as well as maternally. But the infected are not at fault.

Take the real case of 67-year-old Dr D. Basu (name changed) from Hubli. A CT scan for mild abdominal discomfort revealed a 5 cm liver cancer. He had tested HBV-positive 35 years ago, but never followed up or took treatment. He even hid the diagnosis from his family. When we treated his cancer, his brother and 37-year-old daughter Kalyani also tested positive, likely due to maternal transmission and later sexual or vertical transmission. This tragic case underlines some key precautions to follow.

Periodic monitoring in the HBV-infected is a must—one in 10 of those infected develop cirrhosis or cancer. Patients undergo repeated testing but get no treatment as current guidelines are restrictive, excluding nearly 60 per cent from treatment. Patients live with anxiety, fear and stigma. We must move from treating a select few to treating all HBV-positives. One pill, like tenofovir, taken daily can suppress the virus lifelong, halt transmission and prevent complications.



Illustration by RAJ VERMA

ONE PILL, LIKE TENOFOVIR, TAKEN DAILY CAN SUPPRESS THE VIRUS LIFELONG, HALT TRANSMISSION AND PREVENT COMPLICATIONS

Kalyani, Dr Basu's daughter, could have been protected with a birth dose. She was not given HBV vaccination at birth. India's birth-dose coverage is just 63 per cent. Despite being one of the cheapest vaccines and India being a major vaccine producer, the HBV vaccine is scarcely available in private markets here and public awareness is dismal. Only 4.4 per cent of Delhi's adults are fully vaccinated. Are you? Sadly, nearly one-third of health professionals aren't either. Every Indian child and adult should know: the HBV vaccine is safe, effective and for everyone.

COVID taught us the power of self-testing. All close contacts of the HBV-infected must be screened. Enact anti-discrimination laws in education, workplaces and healthcare. We need HBV kits to enable one-stop testing and treatment. At least 80 per cent of diagnosed patients must be linked to free, accessible treatment. Integrating HBV services with programmes for prenatal care, non-communicable diseases, HIV and tuberculosis is essential. We must integrate hepatitis testing into routine healthcare check-ups and general population-based screening. Pregnant women should be screened for both HBV and Hepatitis C Virus (HCV)—the latter spreads only through blood contact and affects 5.5 million Indians.

India launched the National Viral Hepatitis Control Program (NVHCP) on July 28, 2018—one of the world's largest campaigns aiming to eliminate viral hepatitis by 2030. It offers free diagnosis and treatment for HBV and HCV. While impressive work on screening has been done, the treatment uptake remains low. We need a massive awareness drive and a movement for public participation.

With around 3,500 global hepatitis deaths daily—11 per cent from India—the crisis demands urgency. Every avoidable death is a call to act. Each preventable infection highlights our collective failure, and an opportunity to act. We need a society-wide approach, powered by political will, scientific leadership and strong community engagement. Let us break it down. No shame. No blame. Just a cure. ■

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